

hospitals fall under the network category. “The estimate is that there are over 40,000 private hospitals in India. Of these, insurance companies are present only in 20,000-25,000,” says Girdhar J. Gyani, Director General of the Association of Healthcare Providers in India, a body representing private hospitals.

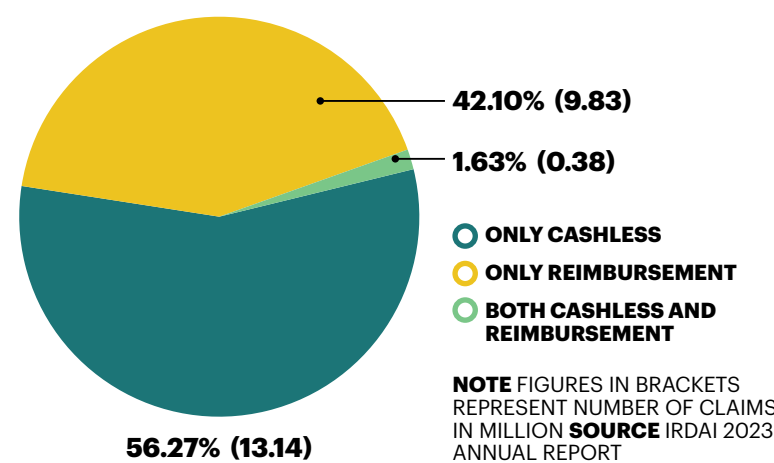
Irdai’s latest annual report shows that during 2022-23, general and health insurers settled 23.36 million health insurance claims with a total payout of ₹70,930 crore. And 56% of the claims were settled through the cashless mode, 42% through the reimbursement mode, and 2% were settled through a combination of the cashless and reimbursement modes.

Sarbvir Singh, Joint Group CEO of insurance platform PB Fintech, says, “The National Health Claims Exchange will significantly aid in facilitating the exchange of health claim information between payers, providers, beneficiaries, and other relevant entities. On an average, every health insurer has a tie-up with about 10,000-15,000 hospitals for cashless treatment. However, customers can also raise requests for treatment in other hospitals with this initiative.” The National Health Claims Exchange, to be set up by the Union health ministry and Irdai, is a digital platform that will bring together insurance companies, healthcare sector service providers, and government insurance scheme administrators.

Even with these initiatives, providing cashless treatment at any hospital is challenging. Currently, major cities have a network of hospitals, but rural and semi-urban areas need support. “Nationwide implementation would require partnerships between insurers and hospitals. Standardised procedures and efficient verification processes are essential, enhancing access and

STATUS CHECK OF CLAIMS

● In total, 23.36 million claims were settled in FY23. The regulator is pushing for a cashless facility ● 56% of health claims were settled through cashless mode, 42% through reimbursement mode, and 2% through both cashless and reimbursement mode last financial year



reducing out-of-pocket expenses,” adds Rakesh Goyal, Director of broker Probusinsurance.com.

However, a cashless facility doesn’t mean you don’t have to pay anything from your pocket. Health insurance policies have certain terms and exclusions. You must read them carefully. Both hospitals and insurers might need to upgrade their technology infrastructure. Secondly, health insurance policies need to reduce exclusions, co-payments, deductions, etc., to make policies more attractive.

Rupinderjit Singh, Senior Vice President of Retail Health at online insurance provider ACKO, says, “Cashless Everywhere is being run to have a common cashless network across health insurers. This should come by the middle of this financial year. However, as of now, only a handful of hospitals are enabled with cashless facilities.”

Experts propose stricter enforcement by Irdai, coupled with incentives for hospitals to join networks. Policyholders need

greater transparency—a clear understanding of network hospitals, covered procedures, and potential out-of-pocket expenses.

They feel the proposed reforms would require a unified list of hospitals compiled by all insurers. A centralised agreement with hospitals should be established through the General Insurance Council, including all licenced insurers. This can ultimately grant every policyholder access to a single, integrated system, streamlining claims processing. Maintaining cashless infrastructure would require investment from insurers, hospitals, and technology providers, which can lead to a rise in premiums.

The insurance industry, which has registered a compound annual growth rate of 10.3% in the last 10 years, could perhaps take the lead.

The stakeholders must focus now on ensuring that the shield of cashless claims settlement facility reaches every citizen soon. **BT**

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